ADULT CARDIAC CATHETERISATION LABORATORY

GUIDELINES 2025
EUROPEAN RESUSCITATION COUNCIL©

1. Prevent & be prepared



Promote adequate training of the staff in technical skills & ALS



Ensure emergency equipment is functional and readily available



Plan elective procedures carefully and use safety checklists

2. Detect and react



Check patient's status and monitored vital signs regulary



Consider echocardiography if haemodynamic instability or complication



In case of cardiac arrest, call for help & activate the resuscitation team

3. Resuscitate and treat possible causes

VF / pVT cardiac arrest



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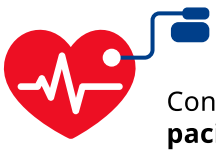
Defibrillate
(apply up to 3 consecutive shocks)

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No ROSC

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Asystole / PEA



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Consider **external or transvenous pacing** in extreme bradycardia

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No ROSC

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Continue CPR according to the ALS algorithm

- Check & correct reversible causes, including **ultrasound & angiography**
- Consider **mechanical CPR** to facilitate chest compression
- Consider **ECPR** in refractory cardiac arrest
- Consider **circulatory support devices** in case of shock after ROSC

ALS – advanced life support; CPR - cardiopulmonary resuscitation; ECPR - extracorporeal cardiopulmonary resuscitation; VF – ventricular fibrillation; pVT – pulseless ventricular tachycardia; ROSC – return of spontaneous circulation; PEA – pulseless electrical activity